

THE PERSONAL INJURY LIFECYCLE: CASE MANAGERS ROADMAP

Master Operational Guide: Compliance, Tech Ecosystems, Liability Systems, and Asset Pipelines

This master operational handout serves as an end-to-end framework mapping out Case Management Systems (CMS), verification portals, structural liability parameters, first-party coverage mechanics (PIP/MedPay), property damage recovery, core firm staff collaborations, and chronological file stages across the entire Personal Injury lifecycle. Use this as a foundational roadmap to be built upon in subsequent training manuals.

Stage 1: Compliance Foundations, Retainers & Time Logging

- **The Contingency Fee Retainer:** Every file legally initializes here. Verify a signed, state-bar-compliant Retainer Agreement is executed in Casepeer upon onboarding. This contract establishes fee tiers and secures the Attorney Fee Lien against future settlement proceeds or verdicts.
- **HIPAA Compliance Mandate:** Protecting Protected Health Information (PHI) is a strict federal requirement under HIPAA. Secure an executed, date-specific HIPAA Release Form immediately during onboarding. Never disclose medical statuses or records to third parties without an audited release on file.
- **Billable Hours & Time Tracking:** Case Managers must log operational actions, telephone conferences, and file updates daily within Casepeer. Accurately tracking time ensures precise firm metric logging and preserves a clean record of case development activities.

Stage 2: Tech Platforms, Request Portals & Staff Intersections

- **Casepeer CMS:** The central firm operating system used daily to log file notes, run time clocks, calendar medical deadlines, update provider treatment entries, and track staff pipelines.
- **Evidence Portals (Add123 & LexisNexis):** Verification channels used to pull asset searches, tag records, locate active driver policies, run background metrics, check municipal filings, and perform location trace queries.
- **Document Retrieval Portals (ChartSwap & Hippo):** Dedicated record retrieval engines utilized to request itemized clinic billing ledgers, secure radiology imaging, and request verified medical chart histories.



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- **Federal Payor Gateways (CMS / BCRC Portal):** The mandatory gateway used to report injury files to the Centers for Medicare & Medicaid Services, track conditional payment letters, and monitor statutory Medicare super-lien totals.
- **Firm Staff Hierarchy & Collaboration:** Case Managers act as the central tactical anchor, coordinating workflows between the Intake Specialist (onboarding lead), Legal Assistant (administrative record requests support), Lien Specialist (lien reducer), Litigation Paralegal (court document support), Trial Attorney (legal advisor), and Accountant (trust accounting/disbursement gatekeeper).

Stage 3: Investigative Intake, Multi-Party Action Plans & Coverage Hunting

- **Practice Area Intake Routing:** For Motor Vehicle Accidents (MVA), secure crash exchange files. For Premises Liability (Slip and Fall / Trip and Fall), secure corporate incident reports, surveillance footage preservation demands, and property maintenance logs. For Animal Liability (Dog Bites), identify the owner and request local Animal Control quarantine records.
- **Multi-Party Action Plans (Handling Passengers):** When an MVA involves multiple passengers in the client's vehicle, execute an immediate Passenger Conflict Audit. If the driver shares liability, the firm cannot represent both due to conflict of interest. If liability is 100% on the adverse vehicle, separate companion files are built to monitor treatment profiles simultaneously, ensuring an equitable pro-rata policy splitting model if limits are low.
- **Deep-Dive Insurance Identification:** Map all active insurance layers, including first-party (PIP/MedPay limits) and third-party BI caps. In low BI environments, query vehicle registration data via Add123 to unearth Separate Vehicle Owner Coverage (vicarious liability) or active underlying umbrella policies.
- **The Prior Injuries & History Audit:** Interview the client point-blank regarding historical medical and prior accident backgrounds. Discovering an unlogged pre-existing condition during a defense deposition can derail a case; identifying it at intake allows the firm to frame an objective 'structural aggravation of a baseline condition' from day one, neutralizing standard insurance defense strategies.

Stage 4: Liability Assessment, Tort Elements & Fault Apportionment

- **The Legal Anatomy of Liability:** Case Managers must confirm the file satisfies the four distinct elements of negligence before submission: Duty of Care (obligations to act reasonably), Breach of Duty (failure to meet standards via action or omission), Causation (proximate link between breach and client trauma), and Damages (quantifiable medical/economic losses).
- **Fault Apportionment Systems:** Apply specific state jurisdictional risk models: (1) Pure Comparative Fault (recovery reduced by client's percentage of negligence), (2) Modified

Comparative Fault (recovery barred completely if negligence crosses a 50% or 51% bar threshold), and (3) Pure Contributory Negligence (any client fault of 1% or more completely bars recovery).

- **Liability Action Plans:** For MVAs, extract crash reports and dispatch logs. For Premises files, issue immediate certified Spoliation Letters demanding 2 hours of surveillance video surrounding the event and maintenance logs to prove Actual or Constructive Notice. For Dog Bites, extract history reports from Animal Control to verify strict liability or one-bite thresholds.

Stage 5: Property Damage, Diminished Value & Loss of Use

- **The Operational Divide:** Handle vehicle damage files instantly to prevent administrative bottlenecks. Map third-party property damage limits early, and pivot to first-party Collision coverage if the adverse carrier stalls liability investigations, seeking deductible recovery via subrogation later.
- **Structural Valuation Auditing:** For repairable files, log hidden structural supplements from body shops in Casepeer. For Total Loss vehicles, review carrier appraisal reports (such as CCC ONE) line-by-line to ensure all options, tire conditions, and interior upgrades are perfectly accounted for in the Actual Cash Value (ACV).
- **Advanced Pockets of Value:** Pursue Loss of Use cash-out options based on local daily rental rates if the client does not require a physical rental vehicle. Audit clean-titled, late-model vehicles for Diminished Value claims by ordering certified history searches via Add123 to capture lost market equity post-repair.
- **Weaponizing Physical Impact:** Extract deep frame rails, floor pan buckling, and seat frame bending blueprints from shop supplements to combine into Section 1 of the BI Demand Packet, proving kinetic force transfer and reinforcing medical causation data.

Stage 6: Calendar Management & Client Communication Frameworks

- **Proactive Calendar Management:** Enforce a strict zero-tolerance calendar policy. Every file must contain an active 'Next Action' task window in Casepeer to track statutory deadlines, records orders, and demand expirations. Never allow a file to sit dark without a task prompt.
- **The 30-Day Mandatory Client Contact Protocol:** Execute a mandatory voice-to-voice touchpoint with every active client at minimum every 30 days to log treatment updates, discover secondary clinical providers, and check ongoing wage impacts.
- **Treatment Mapping Coordination:** Logically map client care across sequential intervals: Acute Assessment (Days 1-14) -> Conservative Rehab Phase (Weeks 2-8) -> Advanced Diagnostics/MRIs (Weeks 4-6) -> Specialist Evaluations (Weeks 8+). Audit files bi-weekly to ensure treatment compliance and eliminate care gaps.



Stage 7: No-Fault 'PIP Physics' & Medical Funding Coordination

- **The Emergency Medical Condition (EMC) Gatekeeper:** In statutory states, secure a signed clinical EMC declaration to unlock the full \$10,000 policy. Files lacking an EMC are hard-capped at \$2,500, causing insurance software evaluation engines to automatically tag the injury as a minor soft-tissue strain.
- **The Mandate to Exhaust PIP:** Ensure providers bill first-party PIP immediately to \$0.00. This removes liability set-off reductions, validates diagnostic urgency to evaluation software, and shifts subsequent care into unmitigated, out-of-pocket special damages under a Letter of Protection (LOP), which acts as a direct multiplier on the general damages baseline.

Stage 8: Medical Payments (MedPay) Coverage & First-Party Coordination

- **Contractual No-Fault Support:** MedPay acts as a no-fault contract layer separate from PIP. It carries no restrictive 14-day rule barriers and requires no active EMC signature to access funding limits (\$1,000, \$5,000, or \$10,000 profiles). It universally steps in to pay for the remaining 20% clinical copay left behind by PIP to settle medical provider ledgers at 100% in real-time.
- **Strategic Funding Coordination:** Instruct clinics to establish a dual-billing template (80% to PIP and 20% to MedPay) to insulate the client from collections. If specialized surgical interventions or MRIs are pending, execute a MedPay Hold Letter to preserve liquid first-party cash pools, bypassing conservative billing tracks to fund advanced diagnostic facility fees directly.
- **Subrogation Risk Mitigation:** Unlike PIP, MedPay carriers maintain contractual reimbursement claims against third-party global settle pools. Case Managers must deploy the contractual Made-Whole Defense in low-limit scenarios to block reimbursement claims completely, or enforce a mandatory reduction cut based on our legal procurement fee percentage.

Stage 9: Advanced Surgical & Interventional Injections Pipeline

- **Interventional Pain Procedures:** Track and categorize targeted injection types (Epidural Steroid Injections, Medial Branch Blocks, Radiofrequency Ablations) and secure matching operative logs. Operative notes must document objective pathology to register in adjuster databases.
- **Pre-Operative Case Management:** If surgery is recommended but pending, pull a formal signed surgical narrative from the specialist along with an itemized future cost projection to demand full policy limits based on future care models.
- **Post-Operative (Post-Op) Monitoring:** Coordinate post-op physical therapy schedules immediately to eliminate care caps. Ensure the surgeon documents permanent

anatomical hardware configurations, surgical scar dimensions, and permanent impairment ratings under the AMA Guides.

Stage 10: Demand Architecture, Pre-Suit Affidavits & Attorney Deliberations

- **Algorithmic Valuation Optimization:** Configure demand packets to feed automated appraisal software (Colossus, ClaimIQ) explicit data targets: objective neurological deficits, CPT code line weights, and documented ADL disruptions.
- **Compulsory Document Inclusion Checklist:** Legal Assistant assembles demand components: (1) Certified CPT billing ledgers, (2) Corresponding provider clinical narratives, (3) Standalone raw radiologist report sheets for diagnostic imaging, and (4) Verified employer-signed lost wage forms. Case Manager audits for compliance before submission.
- **Pre-Suit Case Affidavit Matrix:** Deploy targeted legal disclosures before filing suit, including an Affidavit of No Additional Insurance from the defendant, an Affidavit of Sole Owner/Permissive Use, and a client Household Affidavit to anchor stacked coverage hooks.
- **UM/UIM Demand Pivot & Consent:** If BI limits are insufficient, pivot to UM tracks. Crucial Rule: You must request and secure a formal 'UM Waiver' or 'Consent to Settle' from the UM carrier before executing any third-party BI settlement release to preserve first-party rights.
- **Case Manager - Attorney Deliberations & Calibrations:** Before any demand packet is submitted or any final settlement posture is locked in, the Case Manager must execute a formal file deliberation and calibration session with the handling attorney. Use this review to run risk calculations, evaluate target value metrics, identify litigation exposure factors, and verify that all objective data points align perfectly with the core file strategy.

Stage 11: Pre-Suit Settlements & Release Auditing

- **The Verbal Close & Confirmation:** Once a gross offer is accepted, dispatch an immediate confirmation email within 15 minutes to establish a binding paper trail and block insurer management rollbacks.
- **Release Agreement Auditing:** Review the insurance carrier's formal Release document carefully. Ensure it is a 'Limited Release' or 'Proportionate Release' rather than a Global Release if a secondary UM or health insurance subrogation tracking is active, blocking global waiver language that could inadvertently kill secondary claims.
- **The Mandatory Sign-Off Protocol:** Never execute a liability release until the client has signed a comprehensive, line-by-line electronic disclosure sheet confirming they understand their bottom-line take-home net after fees, costs, and medical bill allocations.



Stage 12: Formal Litigation, Mediation & Arbitration Pipelines

- **Filing & Written Discovery:** When pre-suit channels fail, the lawsuit is filed. Case Managers assist the Litigation Paralegal in compiling data for Requests for Production (RFPs) and Interrogatories. All provider bills and clinical notes must match previous disclosures perfectly to prevent defense exploitation.
- **Court-Ordered Mediation Standby:** Parties meet before a certified neutral mediator. Case Managers remain on standby during mediation to execute real-time phone conferences with clinical providers to secure conditional reduction caps if a global settlement number becomes attainable.
- **Binding Arbitration Awards:** In select instances, the case enters private binding arbitration. The private arbiter or panel issues a final Binding Arbitration Award that dictates the absolute closing payout of the file, carrying no avenue for judicial appeal.

Stage 13: Subrogation, Settlement Liens & Statutory Reductions

- **ERISA Self-Funded Plans:** Employer plans backed by federal ERISA mandates hold strict first-priority recovery rights. Demand Plan Documents (Summary Plan Description and Form 5500) to confirm status, then negotiate a voluntary procurement-cost reduction based on firm fees.
- **Private Health Insurance:** Governed by state insurance statutes. Leverage the contractual Made-Whole Doctrine to secure full subrogation waivers if policy limits failed to compensate the client completely.
- **Medicare Super-Liens:** Federal Secondary Payer statutory mandate. Manage payments early via the BCRC portal. Medicare holds a federal super-lien taking priority over all other parties and automatically applies an administrative reduction for procurement fees upon final closing.
- **Medicaid State Claims:** Medicaid recovery is strictly hard-capped by U.S. Supreme Court precedent (Ahlborn/Gallardo) at the specific portion of the settlement explicitly allocated for past medical expenses caps, protecting pain and suffering awards from balance sweeps.
- **Pro-Rata Provider Reductions:** For standard medical clinic liens and LOP balances, enforce the Pro-Rata Mandate using a formal Proportional Reduction Proposal to resolve clinic lien deadlocks equitably based on available fund caps.

Stage 14: Disbursement, Trust Accounting & File Closure

- **Settlement Distribution Sheet (SDS):** Draft the line-item financial distribution ledger, matching penny-for-penny: Gross Settlement – Attorney Fees – Firm Costs – Negotiated Medical Liens/Subrogation = Client Net Recovery. This calculation satisfies the underlying Attorney Lien and matches the closing ledger boundaries.

- **IOLTA Account Reconciliation:** Verify that the carrier check has been deposited into the firm's Interest on Lawyers' Trust Accounts (IOLTA) and completely cleared by the bank. Never disburse funds on an uncleared instrument.
- **Closing Execution & Lien Waivers:** Upon client signature of the SDS, process firm fee transfers and medical payor distributions. Case Managers must secure a signed, final, notarized Lien Waiver and Release form from every medical provider to shield the client and firm from future collection efforts.

Stage 15: Operational Evidentiary Compliance & Cost Matrix

Case Hardening Cost Category (Add123 & Postal Channels):

Query Type / Verification Channel	Firm Cost Rate
Comprehensive Driver's License History Query (Add123)	\$3.42
Certified VIN / Asset Title History Search (Add123)	\$3.42
Municipal Tag and License Plate Registration Search (Add123)	\$1.74
Real-Time Civil Insurance Coverage Verification Query (Add123)	\$3.00

Outbound Document Scale: Regular First Class Mail is billed at \$0.69; Secure Summons Package Priority Delivery is \$8.50; Certified Mail with Return Receipt Requested (Formal Tenders) is \$8.69; Priority Mail with Signature Confirmation (Litigation Invoices) is \$12.20.



Case Managers Master Lifecycle Checklist

- ☐ Compliance Foundations: Verify a signed Contingency Fee Retainer, fully audited HIPAA release, and primary billable hours entries are logged in Casepeer.
- ☐ SOL Verification: Confirm the correct Date of Loss is logged in our system and checked against statutory parameters.
- ☐ Proactive Calendar Tasks: Confirm every file contains an active 'Next Action' task window with zero unassigned gaps.
- ☐ Client Contact Tracking: Verify a voice-to-voice communication event has been executed and logged within the last 30 days.



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- ☐ Multi-Party Passenger Audit: Verify separate companion files are built and conflict checks cleared for passenger action plans.
- ☐ Liability Anatomy Verification: Confirm file evidence establishes Duty, Breach, Causation, and Damages relative to jurisdictional fault rules.
- ☐ Property Damage Supplement Check: Ensure deep structural impact photos and mechanical supplements are indexed to prove kinetic force transfer.
- ☐ MedPay Allocation Check: Verify if an 80/20 split or a formal MedPay Hold is structurally required to fund specialized facility care lines.
- ☐ Demand Document Inclusions: Verify certified billing ledgers, corresponding narratives, raw radiologist reports, and employer wage verifications are compiled before packet shipment.
- ☐ Attorney Calibration Check: Confirm a formal file review and calibration session has been executed with the attorney prior to demand delivery.
- ☐ Advanced Procedures Tracking: Ensure injection operative logs, surgical narratives, pre-op cost models, and post-op therapy notes are indexed within the architecture.
- ☐ Affidavit Verification Check: Ensure the proper Affidavit of No Additional Insurance or Household Affidavit is executed to secure hidden policy layers.
- ☐ Lien & Statutory Subrogation Audit: Identify health payor parameters (ERISA, Medicare, Medicaid, Private) and order updated audit ledgers to eliminate unrelated charges.
- ☐ Lien Control & Waivers Map: Confirm clinic liens are mapped and that final provider release and waiver receipts are signed and stored.
- ☐ IOLTA Reconciliation: Ensure the settlement funds have completely cleared the firm's trust account prior to printing disbursement checks.